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Abstract

Before Family Risk Becomes a Health Crisis: Trustworthy Premarital Health Education as Primary Prevention Infrastructure

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Background

A family should be a space of physical, psychological, and economic safety. Whether it becomes one is shaped at the transition into marriage, a governance threshold where consent, authority, help-seeking, and access to protection are organized before a household is formed. When that threshold fails, the health consequences include violence-related injury, psychological distress, reproductive harm, and avoidable crisis-service use. Prevention frameworks such as WHO's RESPECT cover relationship skills, empowerment, safe environments, and services, yet this transition remains an underdeveloped intervention point.

Objective

To propose Trustworthy Premarital Health Education (T-PHE) as primary prevention infrastructure that increases each participant's control over marriage-related health decisions within safeguarding and referral boundaries.

Methods

This practitioner-led, document-informed conceptual synthesis drew on the governance and safeguarding procedures of ARAYA Nikah, a Muslim-led social enterprise registered in Thailand, and on its premarital education programme for Muslim and intercultural couples (over 600 participants since 2024). Recurring concerns in these sources were synthesized, through a culturally rooted ethic of limited knowledge, entrusted responsibility, human dignity, just boundaries, and repair, into a programme theory with six safeguards, a failure criterion, and outcome domains for prospective testing. Sources came from one organization; no effectiveness testing was undertaken.

Results

T-PHE is trustworthy only when it protects informed choice rather than relationship continuation. It rests on one mechanism: increasing each participant's control over whether to proceed with,

delay, or refuse marriage and over when and how to seek help. Six safeguards protect that choice: epistemic humility, cultural intelligibility, affected-person visibility, dignity and consent, bounded authority and referral, and repairability. Educators must make power asymmetries and coercion visible, keep culture and religion from becoming unchecked authority, and refer beyond their own competence. Communication and conflict training is not universally benign. Where violence, fear, or coercive control is suspected or disclosed, joint work must stop pending confidential assessment and must not resume where coercive control is confirmed; survivor-centred support and qualified referral take precedence. T-PHE fails if it raises knowledge scores, attendance, satisfaction, or marriage completion while leaving informed choice, recognition of coercion, reproductive and mental-health literacy, safe help-seeking, and accountable referral unchanged.

Conclusion

Premarital health education should be recognized, governed, and tested as primary prevention infrastructure, not optional relationship advice. Public health should enter before the family becomes a site of clinical rescue.

Keywords: premarital health education, health promotion, intimate-partner harm, primary prevention, safeguarding